Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 0.1 | **Date:** 03-Apr-2026

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1. Administrative & Study Identification

Full Official Title: A Phase IIb, Randomised, Multicentre, Double-Blind Study to Evaluate the Effect of Baxdrostat in Combination With Dapagliflozin Compared With Baxdrostat on Albuminuria in Participants With Chronic Kidney Disease and High Blood Pressure. **Study Title:** A Phase IIb Study to Evaluate the Effect of Dapagliflozin in Combination With Baxdrostat Compared With Baxdrostat on Albuminuria in Participants With Chronic Kidney Disease and High Blood Pressure. **Short Title/Acronym:** BaxDuo-Baltic **Protocol Number:** D6972C00006 **NCT Number:** NCT07222917 **Sponsor / Company Name:** AstraZeneca **Investigational Product:** Baxdrostat in combination with Dapagliflozin **Phase of Development:** PHASE2 (Phase IIb) **Trial Status:** NOT_YET_RECRUITING (Not yet recruiting participants) **Medical Monitor:** AstraZeneca Clinical Study Information Center

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the effect of baxdrostat in combination with dapagliflozin compared with baxdrostat/placebo on albuminuria, as measured by the change from baseline in Urine Albumin-to-Creatinine Ratio (UACR) at 12 weeks. **Secondary Objectives:** To determine whether baxdrostat/ dapagliflozin is superior to baxdrostat/placebo at reducing albuminuria, and to assess the general safety and tolerability of the combination.

2.2 Background & Rationale Excess aldosterone is a key driver of cardiovascular, renal, and metabolic injury, promoting hypertension, proteinuria, and progressive chronic kidney disease (CKD). Next-generation aldosterone synthase inhibitors (ASIs), such as baxdrostat, demonstrate high selectivity, enabling potent aldosterone suppression while maintaining cortisol biosynthesis. This study aims to evaluate whether combining baxdrostat with dapagliflozin (an SGLT2 inhibitor) can provide an additive or synergistic effect in improving kidney health and reducing albuminuria in CKD patients with high blood pressure, potentially offering a more effective management pathway.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled (Baxdrostat/Placebo arm) **Blinding:** Double-blind **Randomization:** Randomised (with stratification and capping linked to Type 2 Diabetes Mellitus status)

3.2 Planned Enrollment & Duration Targeted Enrollment: 218 evaluable patients **Number of Sites:** Multicenter/Global (including Europe, Canada, US, Argentina, Taiwan, Thailand, UK, Spain, Bulgaria, Ukraine, Turkey, and South Korea) **Estimated Study Start:** 05-Dec-2025 **Estimated Primary Completion:** 24-May-2027

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants of any sex and gender must be ≥ 18 years of age. 2. Confirmed diagnosis of CKD with $eGFR \ge 30$ and < 90 mL/min/1.73 m2 at screening. 3. Participants with UACR > 200 mg/g (22.6 mg/mmol) and < 5000 mg/g (565 mg/mmol) at screening. 4. Participants with a history of HTN and a Systolic Blood Pressure (SBP) ≥ 130 mmHg at screening and ≥ 120 mmHg at the randomization visit.

4.2 Exclusion Criteria 1. Systolic blood pressure > 180 mmHg, or diastolic blood pressure > 110 mmHg at screening. 2. Known hyperkalemia, defined as potassium of ≥ 5.5 mmol/L within 3 months before screening. 3. Type 1 Diabetes Mellitus (T1DM), or uncontrolled Type 2 Diabetes Mellitus at screening (HbA1C $> 10.5\%$). 4. Any acute kidney injury within 3 months prior to the screening visit.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Drug Name: dapagliflozin **ATC Code:** A10BK01 **Trade Names:** Qternmet, Qtern, Xigduo, Farxiga **Dosage/ Frequency:** One dose of baxdrostat and one standard dose of dapagliflozin (or matching placebo for dapagliflozin) administered daily. **Route of Administration:** Oral **Duration of Treatment:** 12 weeks of exposure.

5.2 Concomitant & Prohibited Therapy Permitted: Stable standard of care for hypertension and CKD (e.g., ACE inhibitors, ARBs). **Prohibited:** SGLT2 inhibitors (participants being treated with SGLT2i must undergo a 2-week washout period prior to randomization).

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Weeks -4 to 0	Optional pre-screening (eGFR, UACR, K+, Na+, BP), 2-week washout of SGLT2i if applicable, Informed Consent
Baseline	Day 0	Randomization, First Dose
Interim	Weeks 2-8	Efficacy Assessment, Safety Labs (Potassium monitoring), BP Checks
End of Study	Week 12	Final Vital Signs, Primary Endpoint UACR Assessment, IP Accountability

(Note: An additional 4-week safety follow-up period generally occurs post-treatment).

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Change from baseline in Urine Albumin-to-Creatinine Ratio (UACR) at Week 12. **Measurement Method:** Central Laboratory Analysis of urine samples.

7.2 Secondary & Exploratory Endpoints * Safety and tolerability profile of the combination. * Change from baseline in Systolic and Diastolic Blood Pressure. * Measurements of specific parameters including potassium and sodium changes.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Collection via HCP assessment, laboratory monitoring (with strict potassium and sodium checks), and patient reporting. **Serious Adverse Events (SAEs):** Standard protocol timeline reporting (typically 24 hours to Sponsor). **Data Monitoring Committee (DMC):** External/Internal independent safety monitoring.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy analysis, Safety Set for all AE/SAE evaluations. **Sample Size Justification:** Target enrollment of 218 subjects required to detect a statistically significant reduction in UACR for the combination arm versus baxdrostat alone. **Handling of Missing Data:** Mixed-Model Repeated-Measures (MMRM) or Multiple Imputation for missing continuous variables like UACR.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits by Clinical Research Associates (CRAs). **Audit Trail:** Electronic Case Report Form (eCRF) utilizing a compliant EDC system with full audit trails and data clarification mechanisms.

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11. Study Identifiers & Governance

Primary ID: D6972C00006 **Posting ID:** 568220845953007107 **Registry Number:** NCT07222917 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** A Phase IIb Study to Evaluate the Effect of Dapagliflozin in Combination With Baxdrostat Compared With Baxdrostat on Albuminuria in Participants With Chronic Kidney Disease and High Blood Pressure.

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12. Clinical Framework (Hyperkalemia/CKD Specific)

Disease Areas / Name: Chronic Kidney Disease and Hypertension **Preferred Name:** Chronic kidney disease due to hypertension **Preferred UMLS Name:** Chronic Kidney Disease **Diagnosis Threshold:** $eGFR \ge 30$ and < 90 mL/min/1.73 m2; UACR > 200 mg/g; SBP $ \ge 130$ mmHg **Medical Methodology:** Aldosterone Synthase Inhibition combined with SGLT2 inhibition. **Optimization Target:** Reduction of albuminuria (UACR) and blood pressure control.

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13. Study Procedures & Methodology

Management Pathway: Standard of care background therapy combined with investigational oral daily dosing. **Education Protocol (HCP):** Investigator training detailing protocol-specific UACR measurement, BP tracking, and SGLT2i washout criteria. **Education Protocol (Patient):** Counseling on trial drug adherence, BP monitoring, and dietary instructions. **Quality Audit Mechanism:** Centralized laboratory data review and continuous data quality checks.

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14. Observation & Follow-Up Schedule

Total Duration: Up to 18-20 weeks (up to 4-week screening/washout + 12-week treatment + 4-week follow-up). **Onsite Visit Cadence:** Pre-screening, Screening, Baseline (Day 0), and periodic visits throughout the 12-week treatment phase. **Remote Monitoring Frequency:** As per the study-specific clinical monitoring plan. **Checklist Review Interval:** Routine eCRF data entry and verification.

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15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- | | **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | | Clinical Operations Lead | [Name] | ___ |** ***[DD-MMM-YYYY] | | Lead Statistician | [Name] |*** _____ | [DD-MMM-YYYY] |